**Clinical Study Summary Report (CSSR)**
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## 1. Administrative & Study Identification

**Full Study Title:** A Phase I, Multicenter, Open-label, First-in-Human, Dose Escalation and Expansion Study of AZD9592 as Monotherapy and in Combination With Anti-cancer Agents in Patients With Advanced Solid Tumors
**Short Title/Acronym:** First in Human Study of AZD9592 in Solid Tumors (EGRET)
**Protocol Number:** D9350C00001
**NCT Number:** NCT05647122
**Posting ID:** 920616827708809047
**Trial Status:** RECRUITING
**Sponsor Name:** AstraZeneca (Company: AstraZeneca)
**Investigational Product:** AZD9592 (bispecific ADC targeting EGFR and c-MET), Osimertinib, fluorouracil (5-Fluorouracil, ATC Code: L01BC02; Trade names: Carac, Efudex, Adrucil, Fluoroplex, Tolak), Bevacizumab, Leucovorin
**Preferred UMLS Name:** Antineoplastic Agents
**Phase of Development:** Phase I (PHASE1)
**Medical Monitor:** AstraZeneca Clinical Operations Lead

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## 2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** * **Part A (Dose Escalation):** To assess safety and tolerability, and to determine the maximum tolerated dose (MTD) and/or recommended phase 2 dose (RP2D) of AZD9592 as monotherapy or in combination with osimertinib. * **Part B (Dose Expansion):** To evaluate safety, tolerability, and antitumor activity by investigator-assessed objective response rate (ORR) according to RECIST v1.1.

**Secondary Objectives:** To assess AZD9592, as monotherapy and in combination with osimertinib, for efficacy (by ORR, disease control rate [DCR] at 12 weeks, duration of response, progression-free survival, and overall survival), pharmacokinetics (PK), and immunogenicity.

**2.2 Background & Rationale** To address the need for targeted therapies in advanced solid tumors expressing EGFR and c-MET. AZD9592 is a novel antibody-drug conjugate (ADC) with a bispecific antibody targeting EGFR and c-MET. When internalized, AZD9592 releases a topoisomerase 1 inhibitor (TOP1i) warhead that induces apoptotic cell death. Preclinical data have demonstrated promising efficacy and safety in tumors expressing these receptors, including non-small-cell lung cancer (NSCLC), head and neck squamous cell carcinoma (HNSCC), and colorectal cancer (CRC).

## 3. Study Design & Methodology

**3.1 Design Framework Study Type:** Interventional **Control Type:** Single-arm (Dose escalation and dose expansion cohorts per module) **Blinding:** Open-label **Randomization:** N/A

**3.2 Planned Enrollment & Duration Target $N$:** 403 **Number of Sites:** Multicenter (Including sites across the EU, USA, and Asia Pacific) **Estimated Study Start:** 21-Dec-2022 **Estimated Primary Completion:** 06-Oct-2027

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## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Age $\ge 18$ years, with Eastern Cooperative Oncology Group (ECOG) Performance Status of 0-1, and life expectancy $\ge 12$ weeks. 2. Measurable disease per RECIST v1.1 and adequate organ and marrow function. 3. Histologically or cytologically confirmed metastatic or locally advanced EGFRmut or EGFRwt NSCLC, recurrent or metastatic HNSCC, or metastatic CRC (depending on the study module).

**4.2 Exclusion Criteria** 1. History of (non-infectious) ILD/ pneumonitis that required steroids, current ILD/pneumonitis, or where suspected ILD/pneumonitis cannot be ruled out. 2. Unresolved toxicities of Grade $\ge 2$ (NCI CTCAE v5.0) from prior therapy, or previous treatment with a TOP1i ADC or an EGFR- or MET-targeted ADC. 3. Spinal cord compression, history of leptomeningeal carcinomatosis, active infection (including tuberculosis, HBV, HCV, or HIV), or untreated/unstable brain metastases requiring continuous corticosteroids.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Conducted in multiple modules: Module 1 (AZD9592 Monotherapy), Module 2 (AZD9592 + Osimertinib), and Module 3 (AZD9592 + fluorouracil/5-FU, Bevacizumab, Leucovorin). **Route of Administration:** Intravenous (IV) infusion for AZD9592, fluorouracil/5-FU, Bevacizumab, and Leucovorin; Oral (PO) for Osimertinib. **Duration of Treatment:** Treatment continues until disease progression, initiation of alternative anticancer therapy, or unacceptable toxicity.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care for managing disease-related symptoms. **Prohibited:** Alternative anticancer therapies, previous TOP1i ADCs, or EGFR/MET-targeted ADCs.

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## 6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures |
| :--- | :--- | :--- |
| **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor Imaging (RECIST v1.1), Safety Labs |
| **Baseline** | Day 0 | First Dose, Baseline PK/PD/Immunogenicity Sampling, Safety Assessments |
| **Interim** | Per Cycle | Safety Monitoring, Efficacy Assessment (Tumor Scans), Dose-Limiting Toxicity (DLT) Evaluation for Part A |
| **End of Study** | Progression | Final Safety Labs, Exit Interview, Survival Follow-up |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** * **Part A:** Incidence of Adverse Events (AEs), Serious Adverse Events (SAEs), and Dose-Limiting Toxicities (DLTs) to establish MTD and RP2D. * **Part B:** Investigator-assessed Objective Response Rate (ORR). **Measurement Method:** Clinical assessment using NCI CTCAE v5.0 for toxicities and RECIST v1.1 for tumor response.

**7.2 Secondary & Exploratory Endpoints** Disease Control Rate (DCR) at 12 weeks, Duration of Response (DoR), Progression-Free Survival (PFS), and Overall Survival (OS). Pharmacokinetics (PK) and immunogenicity profiles.

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Continuous monitoring of AEs and DLTs graded per NCI CTCAE v5.0. **Serious Adverse Events (SAEs):** Standard SAE reporting protocols applied throughout the study duration. **Data Monitoring Committee (DMC):** Safety data review by internal Dose Escalation Committee to clear dose levels before subsequent cohort expansions.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Safety analysis set for patients receiving $\ge 1$ dose of study treatment; Efficacy analysis set. **Sample Size Justification:** $N = 403$; appropriately sized for a modular Phase I dose escalation (Part A) and dose expansion (Part B) trial to detect preliminary safety and efficacy signals. **Handling of Missing Data:** Descriptive statistics will be used for all variables without general imputation of missing data for safety and PK endpoints.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine clinical site monitoring for Protocol and GCP adherence. **Audit Trail:** Secure Electronic Data Capture (EDC) systems utilized for eCRF locking and compliance validation.

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## 11. Study Identifiers & Governance

**Primary ID:** {{D9350C00001}} **Posting ID:** {{920616827708809047}} **Registry Number:** {{NCT05647122}} **Trial Status:** {{RECRUITING}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Company:** {{AstraZeneca}} **Study Title:** {{First in Human Study of AZD9592 in Solid Tumors (EGRET)}} **Official Regulatory Title:** {{A Phase I, Multicenter, Open-label, First-in-Human, Dose Escalation and Expansion Study of AZD9592 as Monotherapy and in Combination With Anti-cancer Agents in Patients With Advanced Solid Tumors}}

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## 12. Clinical Framework & Ontology Data

**Primary Condition:** {{Advanced Solid Tumours, Carcinoma Non-small Cell Lung, Head and Neck Neoplasms, Colorectal Neoplasms}} **Condition Ontology Name:** {{Carcinoma (Preferred Name: Carcinoma)}} **Semantic Type:** {{Neoplastic Process}} **Condition Definition:** {{A malignant tumor arising from epithelial cells. Carcinomas that arise from glandular epithelium are called adenocarcinomas, those that arise from squamous epithelium are called squamous cell carcinomas, and those that arise from transitional epithelium are called transitional cell carcinomas (NCI Thesaurus). [https://orcid.org/0000-0002-0736-9199]}} **Ontology Codes:** {{MeSH: D002277 | HPO: HP:0030731 | SNOMED: 1187425009}} **SNOMED Hierarchy:** {{1187225007|Malignant epithelial neoplasm (morphologic abnormality), 118285006|Epithelial neoplasm (morphologic abnormality), 108369006|Neoplasm (morphologic abnormality), 1240414004|Malignant neoplasm (morphologic abnormality)}} **Diagnosis Threshold:** {{Measurable disease per RECIST v1.1}} **Medical Methodology:** {{Antibody-Drug Conjugate (ADC) bispecific targeted therapy (EGFR and c-MET), Antineoplastic Agents}} **Optimization Target:** {{Maximum Tolerated Dose (MTD) & Recommended Phase 2 Dose (RP2D)}} **Source Level:** {{5}}

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## 13. Study Procedures & Methodology

**Management Pathway:** {{Modular dose escalation (Part A) followed by dose expansion (Part B)}} **Education Protocol (HCP):** {{Investigator training on grading and managing ADC-related toxicities (e.g., ILD/pneumonitis)}} **Education Protocol (Patient):** {{Informed consent and active symptom monitoring/ reporting}} **Quality Audit Mechanism:** {{Centralized safety review by clinical team to approve dose cohort escalation}}

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## 14. Observation & Follow-Up Schedule

**Total Duration:** {{Until disease progression, unacceptable toxicity, or study closure; followed by survival follow-up}} **Onsite Visit Cadence:** {{Standard Phase I interval monitoring (per protocol cycles)}} **Remote Monitoring Frequency:** {{As per specific site remote-monitoring policies and patient safety tracking}} **Checklist Review Interval:** {{Prior to each dose escalation review window}}

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## 15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY] | | Lead Statistician | [Name] |*** _____ | [DD-MMM-YYYY] |